Skanda Radiology

RADIOLOGY REPORT — AI-ASSISTED DRAFT

Report date: 10/2/2026 · Accession: C9F4BA7B08

Patient Name	**Demo Patient**	Patient ID	—
Age / Sex	**58 / Male**	Study Date	**2026-10-02**
Referring Physician	—		

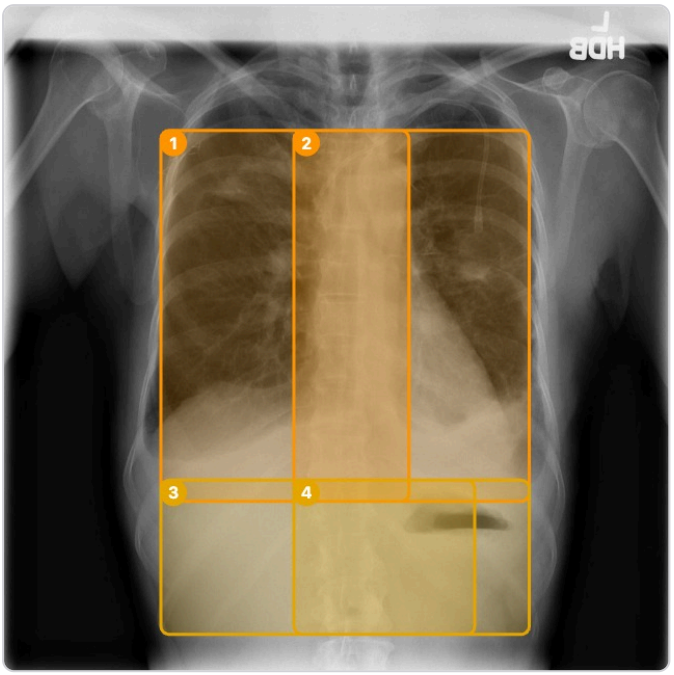
CLINICAL INDICATION / HISTORY

Cough and breathlessness for 2 weeks

EXAM

Radiograph (X-ray) · Chest · PA

KEY IMAGES

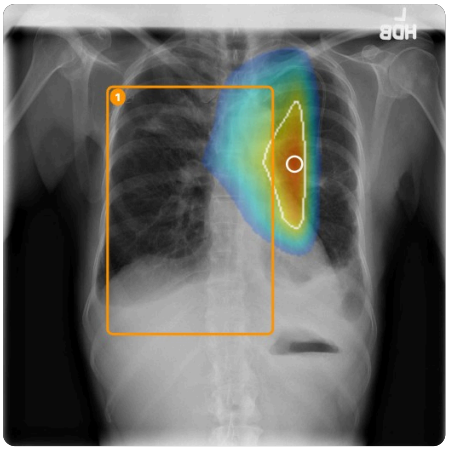


1. **Left lower lobe consolidation** · left lower zone
2. **Right lower lobe consolidation** · right lower zone
3. **Left pleural effusion** · left base
4. **Right pleural effusion** · right base
5. **Increased bronchovascular markings / perihilar opacity** · Perihilar regions, bilateral · not localized

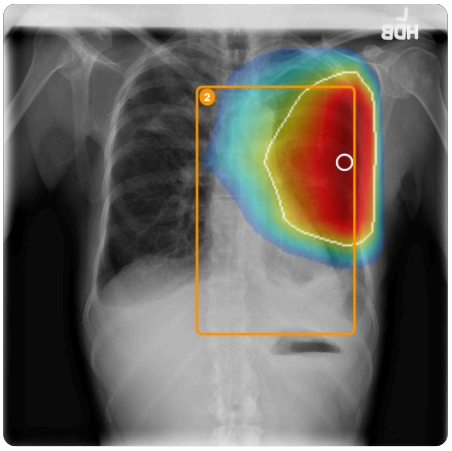
Key 1 — cxd_45707.jpg

CLASSIFIER HEATMAPS

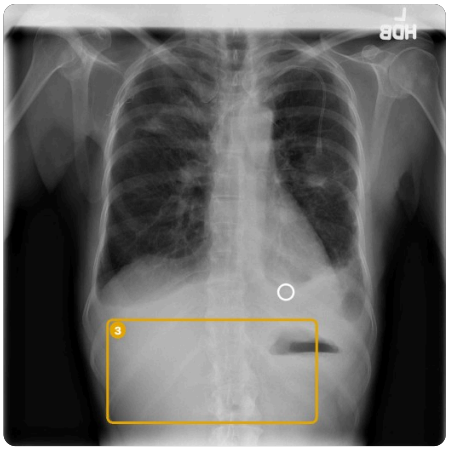
One map per finding: where an independent image classifier (TorchXRayVision) found evidence for that finding type, within the finding's own region (its box, or the zone it names). Red = strongest, outlined = hot region, ring = peak; the box is the reading model's. "Regional evidence" is the strongest value in the region relative to the type's peak anywhere on the film: low means the classifier sees little at that location. Coarse: on held-out radiologist-labelled films the peak fell inside the lesion 17% of the time. A prompt to look, not a localisation.



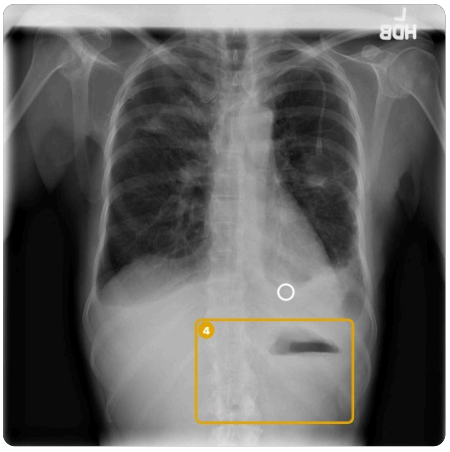
1 Left lower lobe consolidation
opacity · classifier 73% · supported · regional evidence 78%



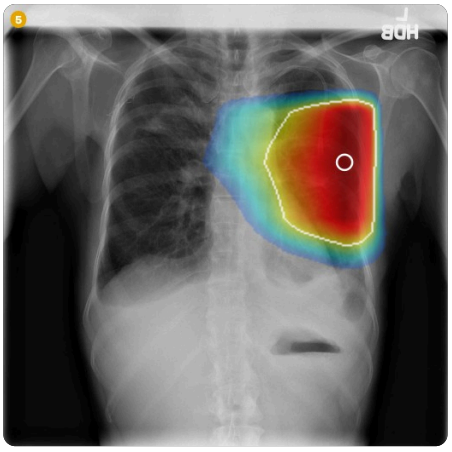
2 Right lower lobe consolidation
opacity · classifier 73% · supported · regional evidence 100%



3 Left pleural effusion
effusion · classifier 75% · supported · regional evidence 19%



4 Right pleural effusion
effusion · classifier 75% · supported · regional evidence 19%



5 Increased bronchovascular markings / perihilar opacity
opacity · classifier 73% · supported · regional evidence 100%

TECHNIQUE

PA Chest Radiograph

COMPARISON

None available.

FINDINGS

There are bilateral lower lobe consolidations, obscuring the diaphragmatic contours and cardiac border on the left, and the hemidiaphragm on the right. These findings suggest pneumonia or other infiltrative processes affecting both lower lobes.

Small to moderate pleural effusions are present bilaterally, indicated by blunting of the costophrenic angles on both sides (left and right base) and increased opacity along the lateral chest walls.

There is evidence of increased bronchovascular markings and possible peribronchial cuffing, particularly in the lower lobes, suggesting interstitial changes or inflammation.

The heart size appears within normal limits. The mediastinum is midline. There is no pneumothorax identified.

IMPRESSION

1. **Bilateral lower lobe consolidations and pleural effusions are present, requiring clinical correlation to determine the etiology (e.g., infection, malignancy, fluid overload).**
2. **Increased bronchovascular markings suggest interstitial involvement.**
3. **No pneumothorax or significant mediastinal abnormality identified.**

RECOMMENDATIONS

Clinical evaluation for cough and breathlessness is warranted to determine the cause of the bilateral consolidations and effusions.

Consider CT chest imaging for further characterization of the lung opacities, pleural effusions, and interstitial changes if clinically indicated (e.g., failure to respond to initial treatment, suspicion of malignancy).

Follow-up chest radiography may be considered after treatment initiation to assess response.

AI FINDINGS SUMMARY

#	FINDING	LOCATION	SEVERITY	BENCHMARK	MODEL-STATED (UNCALIBRATED)
1	Left lower lobe consolidation	left lower zone	Significant	29/38	95%
2	Right lower lobe consolidation	right lower zone	Significant	29/38	95%
3	Left pleural effusion	left base	Moderate	12/20	85%
4	Right pleural effusion	right base	Moderate	12/20	85%
5	Increased bronchovascular markings / perihilar opacity	Perihilar regions, bilateral	Moderate	29/38	60%

Benchmark: how many calls of this kind were confirmed by reference labels in the local benchmark. Model-stated confidence is the model's own figure and is not calibrated.

UNSIGNED — PRELIMINARY AI DRAFT

Awaiting radiologist review and signature

AI-generated decision support; must be verified by a qualified radiologist. Not a diagnosis. Drafted with AI assistance (MedGemma 27B, run locally).